Ch 105 · Interdisciplinary care and communication

Team benefits

- Multidisciplinary rounds ↓ mortality; pharmacist ↓ drug errors; checklists improve compliance

Communication and safety

- SBAR / I-PASS handover; daily goals
- Psychological safety: speak-up culture (CUS, two-challenge rule)
- Crew resource management, simulation, blame-free incident learning

Burnout

- Exhaustion, depersonalisation, low accomplishment; moral distress
- Debriefing, ethics support, staffing, involvement in decisions

Key point: Most ICU errors are communication failures.

Ch 106 · End-of-life care in ICU

Principles

- Respect wishes/advance directives; best interests if no capacity
- Shared decisions; no duty to give non-beneficial treatment; withholding = withdrawing

Managing conflict

- Family meeting (VALUE), explore patient's values, honest prognosis
- Time-limited trial; second opinion; ethics/legal route if needed; document

Withdrawal process

- Comfort-focused opioids/benzodiazepines (double effect)
- Terminal extubation or wean; family presence; organ donation referral; bereavement support

Key point: The question is what the patient would want, not what the family wants.

Ch 107 · Palliative care in ICU

Models

- Integrative (ICU team) and consultative (specialist triggers)
- Triggers: advanced cancer, ICU stay > 10 days, multiorgan failure, conflict

Symptom control

- Pain (opioids + adjuvants), dyspnoea (opioids, fan), delirium, nausea, secretions
- Psychological and spiritual support

Goals of care

- Ask–tell–ask; realistic outcomes; ceilings of care; revisit often

Key point: Palliative care runs alongside active treatment, not after it.

Ch 108 · Family involvement in ICU

Benefits

- Better trust and decisions; less anxiety/PTSD in relatives; helps prevent delirium

Strategies

- Open visiting, participation in rounds and care, early family conference (≤ 72 h)
- ICU diaries, written information, interpreters, consistent communicator

Presence during CPR

- Fewer PTSD symptoms in relatives (Jabre 2013); no effect on CPR quality with a support person

Key point: Families are part of the care team, not visitors.

Ch 109 · Post-intensive care syndrome

Domains

- Physical (weakness, fatigue), cognitive (memory, attention), mental (anxiety, depression, PTSD)
- PICS-F in family members

Risk factors

- Delirium, sepsis, ARDS, long ventilation, deep sedation, benzodiazepines, immobility, frailty

Prevention and recovery

- ABCDEF bundle, early mobility, family engagement; ICU diaries ↓ PTSD
- Follow-up clinics, rehabilitation, return-to-work and peer support

Key point: Survival is not the only outcome – plan recovery from day one.